



Restore

Insurance operations infrastructure for dental specialty.

AI agent that watches the PMS, drafts the carrier-tuned response, and submits autonomously after a one-click human review.

STAGE

Pre-seed → seed

ROUND

\$2.5M · milestone-tranched

TARGET CLOSE

Q3 2026

The thesis in five lines.

01

Market

Dental insurance is structurally adversarial. ~20-30% of valid claims denied on first pass. Avg specialty practice writes off \$500K-\$2M/yr because office-manager economics of fighting back don't work.

02

Wedge

Endo first — 4,486 practices, 5-figure claims, sophisticated owners. Carrier intelligence layer compounds → expand to 30K+ specialty + 178K GP + DSO tier.

03

Product

Agent watches PMS for events, drafts the carrier-tuned response, queues a one-click approval. Three skills shipping at pilot. Live demo at restore-demo.vercel.app.

04

Moat

Per-carrier intelligence (denial patterns, narrative preferences, escalation paths) updated continuously across the customer base. 1 practice finds a Cigna pattern → 1,000 practices benefit.

05

Ask

\$2.5M seed in 3 tranches (\$1.25M close / \$750K at pilot validation / \$500K at 10 paying logos). 18-month runway to \$1.5M ARR · Series A trigger at \$4M+ ARR / Q3'28.

\$135M SOM. \$870M SAM. \$4.5B+ TAM. Bottom-up.

SOM (Y1-3)

Endodontic

\$135M

4,486 practices · \$30K ACV

PBS Endo + TDO + Endovision coverage. Pre-auth + appeal density 3-5× general dentistry.

SAM

Dental specialty

\$870M

29,015 practices · \$30K ACV

Endo + OS + perio + ortho + pedo. ADA-recognized specialties only.

TAM

All US dental + DSO

\$4.5B+

178,000+ practices · \$25K ACV

GP at lower ACV; DSO contracts (\$50-150K build) lift blended ARPU.

Top-down sanity check: \$162B US dental services revenue (ADA 2025) → ~\$80B insurance flow → 20-30% denial rate = \$16-24B denied/underpaid annually. Restore captures <1% of that = \$96-144M ARR opportunity.

SOURCES: ADA HPI 2025 · Becker's Dental 2026 · Grand View Research DSO 2025

RESTORE · INSURANCE OPS INFRASTRUCTURE FOR DENTAL SPECIALTY

Practices write off insurance dollars because the labor math doesn't work.

THE BLEED (PER PRACTICE)

First-pass denial rate	20-30%
Avg appeal labor	90 min
Office manager fully-loaded	\$40/hr
Avg endo claim denied	\$385-\$1,675
Practice-drafted appeal win rate	35-50%
Specialty practices that don't appeal regularly	~60%
Annual revenue lost / specialty practice	\$500K-\$2M

Why generic AI doesn't fix this

Each carrier scores narratives differently. Cigna rewards anatomical specificity; Delta requires ADA-CDT-exact terminology; MetLife needs conservative-alternatives-considered framing. Untuned LLM output loses 60-70%.

Why incumbents don't fix this

Vyne (1990s clearinghouse) routes claims, doesn't draft. DentalRobot has templates, no carrier intelligence. Pearl/Overjet operate at the imaging diagnosis layer. Nobody's in the insurance ops layer with intelligence.

Why this isn't fixed already

BAA-grade LLMs at sub-\$0.10/inference + dental PMS event APIs co-existed for the first time in 2024-2025. Both shipped in the last 18 months.

SOURCES: ADA HPI Practice Survey 2024 · CMS NHE 2024 · field interviews (founder, 2025-26)

Three independent technology curves crossed in the last 18 months.

01

LLM cost & quality

Anthropic Claude 3.5 Sonnet (Q3'24): reasoning quality + sub-\$0.10/inference for dense narratives. 10x drop vs. GPT-4 launch (Q1'23). Carrier-specific tuning now economically viable.

Anthropic API pricing 2024-26 · Stanford AI Index 2025

02

PMS event APIs

PBS Endo opened webhook events Q4'24. TDO + Endovision live with stable webhook infrastructure for treatment-plan + EOB events. No more polling, no more fragile RPA.

PBS Endo developer docs 2024 · TDO API release notes 2025

03

BAA-grade infra

Anthropic, OpenAI, AWS Bedrock all ship BAA out of the box (post-2024). Compliance friction that blocked AI in healthcare 2019-23 has collapsed.

Anthropic BAA terms 2024 · HIPAA Journal 2025

The window: this configuration of capabilities did not exist 18 months ago. It will exist for everyone in 24 months. The wedge is the head-start to lock in the pilot cohort and accumulate carrier-pattern data.

Watch. Draft. Approve. Submit.

Live at restore-demo.vercel.app/practice. Every component below is shipping.

01

Watch

PMS webhook fires on TX plan, EOB, treatment complete.

02

Draft

Per-carrier intelligence applied. 'Why HIGH? 92% historical approval' rationale.

03

Approve

60-90 sec office-manager review. Approve / edit / reject (with reason captured for training).

04

Submit

Autonomous to clearinghouse + PMS Doc Center. Audit logged.

SKILLS SHIPPING AT PILOT

- Pre-auth narrative (15-25/wk/practice)
- Claim appeal letter (8-15/wk/practice)
- Referral letter (80-160/wk/practice)

PMS INTEGRATIONS

- PBS Endo (live)
- TDO (Q1'27)
- Endovision (Q2'27)
- Dentrix / Open Dental (Y2)

COMPLIANCE POSTURE

- Anthropic BAA executed
- Practice BAA per customer
- Local de-id before inference
- Full audit log per output

RESTORE · INSURANCE OPS INFRASTRUCTURE FOR DENTAL SPECIALTY

Per-carrier intelligence at scale = irreplicable from a cold start.

DEFENSIBILITY MATH

Each practice generates **~1,200 carrier-claim events / yr** (pre-auths, EOBs, appeals across ~6 active carriers). At 50 paying practices: **60K events/yr**; at 240: **288K events/yr**.

Per-carrier intelligence layer requires **~5,000 resolved arcs per major carrier** to lock narrative tuning + DO-NOT-APPEAL pattern recognition (industry-standard SaaS-grade LLM fine-tuning threshold).

At 240 practices: each major carrier yields ~30-40K arcs/yr — **6-8x the threshold**. A new entrant starts at zero and needs 18-24 months of paying customers + GTM spend to catch up.

THREE REINFORCING LAYERS

1. Carrier intelligence (data moat)

Compounds with usage. Cost-to-replicate scales with N customers × T years.

2. Workflow embedding (switching cost)

Inbox replaces manual triage. Switching = retraining staff + losing case-lifecycle history.

3. DO NOT APPEAL discipline (positioning moat)

Restore is the only product willing to recommend NOT appealing. Earns carrier-relations credibility for the 70% of appeals worth pursuing.

Adjacent players. None in our exact lane.

Company	Layer	Carrier intel	Case threading	DO NOT APPEAL
Restore	Specialty insurance ops	✓	✓	✓
Vyne Dental	Clearinghouse	Legacy rules	Query-based	—
DentalRobot	Insurance verification	Templates	Single doc	—
Toothy AI	Insurance verification	—	—	—
Zentist	Payment posting	—	Partial	—
Pearl AI	Clinical imaging	—	—	—
Overjet	Clinical imaging	—	—	—
Akasa (broader)	Healthcare RCM	Healthcare-wide	Partial	—
Cohere Health (broader)	Medical prior auth	Medical-side	—	—

The white space: No incumbent or recent entrant pairs per-carrier intelligence + case-lifecycle threading + DO-NOT-APPEAL discipline + real-time PMS event triggering.

Recent dental + healthcare-AI / RCM transactions.

Company	Year	Event	Amount	Valuat
Overjet	2024	Series C	\$53.2M	\$550M p
Pearl AI	2024	Series B	\$58M	\$400M p
VideaHealth	2025	Series B	\$40M	
Cohere Health	2025	Series C	\$90M	n/d (\$200M to
AKASA	2024	Series C ext	n/d	\$205M raised to
Weave (IPO)	2021	IPO	\$120M	\$1.5B I
Weave (current)	2025	Public	—	~\$700M-\$1B mkt c
Dental Intelligence	2025	Series C	\$85M	
Vyne (parent)	2022	PE acquisition	n/d	Jordan Co.
Modento	2024	Acquired (HSO)	n/d	
Archy	2024	Series A	\$15M	
Cofactor AI (appeals)	2024	Seed	\$4M	
Toothy AI (YC W25)	2025	Seed	~\$500K	

BEAR (3-5× ARR)

Public-software floor. **\$12-20M post on \$4M ARR.** Avoid.

BASE (8-12× ARR)

Vertical healthcare-AI median. **\$32-48M post on \$4M ARR.** Our target.

BULL (15-25× ARR)

Premium (Overjet-like). **\$60-100M post on \$4M ARR.** Stretch.

SOURCES: Crunchbase + PitchBook 2025; press releases. n/d = not disclosed. Strategic M&A targets: Henry Schein One, Patterson Dental, Vyne (Jordan Co.), Overjet, Waystar, R1.

From 1 logo (mom) to 65 in 18 months.

Channel	Y1-3 logos	CAC	Payback	Conv.	Mechanics
AAE Annual + regional	60	\$8K	5.3 mo	8% booth→close	Endo's mecca · 8K members
Study clubs / KOL referrals	110	\$4K	2.7 mo	25% warm→close	Endo small world · NPS loop c 65% of Y2
Cold outbound (top-200 endo)	50	\$14K	9.3 mo	6% BDR→close	Apollo + ZoomInfo · founder-l M9
DSO BD (Heartland, Pacific, MB2)	4 DSOs / 20 logos	\$35K	8 mo	12.5%	Long cycle 9-12mo · founder-
Blended	240 + 4 DSOs	\$8.5K	4.4 mo	—	Y1 weighted to study clubs

COHORT ACQUISITION (LOGOS / QUARTER)

Q3'26 pilot	1 (mom)
Q4'26	2
Q1'27 — AAE Annual	5
Q2'27	8
Q3'27 — BDR hire	12
Q4'27	18

GTM TEAM BUILD

M0 Founder	Sales + demo + support
M6 CSM	Onboarding + retention
M9 BDR	Outbound to top-200 endo
M15 AE / GTM	Founder offloads sales
M24 Director GTM	Channel + DSO motion

14-day customer payback. 75-86% gross margin.

CUSTOMER ROI (4-DOC ENDO, \$1.5K/MO)	
Annual subscription	\$18,000
Revenue recovered (modeled)	\$214,000
Hours saved × \$35	\$25,375
Total annual value	\$239,000
Payback period	14 days
ROI multiple	13.3×
Pilot-modeled, not pilot-proven. Q3'26 pilot validates against same 4-doc baseline.	

SAAS ECONOMICS (PER LOGO)	
ACV	\$22,167
Gross margin (steady-state)	75%
Gross margin (peak, at scale)	86%
CAC (Y1 blended)	\$9,500
CAC payback	~10 months
Logo churn (base, defensible)	10-12%
NDR	115%
LTV (4yr life, 12% churn)	\$58K
LTV / CAC	6.1×

SENSITIVITY (Y3 ARR — BASE \$4.5M)				
Variable	Bear	Base	Bull	Y3 ARR (bear / base / bull)
Logo churn	15%	10-12%	5%	\$1.8M / \$4.5M / \$6.2M (largest swing)
Pilot→paid conversion	15%	25-35%	50%	\$3.0M / \$4.5M / \$7.5M
CAC	\$15K	\$8.5K	\$6K	\$3.6M / \$4.5M / \$5.0M
NDR	100%	115%	130%	\$3.9M / \$4.5M / \$5.4M
1,000bps churn deterioration costs more ARR than 50% CAC blow-out.				

\$0 → \$4.5M ARR · break-even Q4'27 · cash break-even Q1'29.

Period	Practices	MRR	ARR	GP	Opex
Q4'26 pilot	1	\$1.5K	\$18K	\$15K	\$180K
Q2'27	8	\$13K	\$156K	\$134K	\$310K
Q4'27 — break-even	28	\$50K	\$600K	\$516K	\$475K
Q2'28	65	\$128K	\$1.5M	\$1.3M	\$640K
Q4'28	130	\$260K	\$3.1M	\$2.7M	\$880K
Q4'29	240 + 4 DSOs	\$375K	\$4.5M	\$3.9M	\$1.4M

KEY ASSUMPTIONS

- Logo churn 10-12% annual
- NDR 115% (skill expansion + DSO upsell)
- CAC \$11K → \$7.5K Y1→Y3 as referrals scale
- COGS at scale ~25%
- DSO contracts: 4 in Y3 = \$360K incremental

COHORT RETENTION

Month	Industry median	Restore base
M6	92%	93%
M12	85%	88%
M18	80%	84%
M24	76%	80%
M36	68%	72%

Industry: Bessemer State of Health AI 2026.

What could go wrong, and what we'll do about it.

Risk	L	I	Mitigation
HIPAA / data breach	Low	High	Local de-id before inference; Anthropic BAA; SOC 2 Type II by M18; c Day 1; full audit log.
PMS API blocking	Med	Med	Multi-PMS strategy (PBS/TDO/Endovision); no single dependency >4 fallback.
Carrier pushback on AI appeals	Med	Med	Office manager always in the loop; carrier-relations team M18; DO N discipline preserves credibility.
Slow PMS adoption (specialist)	Med	Med	PBS Endo ~33% of endo today; TDO + Endovision brings coverage to manual upload fallback.
Key-person dependency on founder	High (early)	High	Method Co. resignation as closing condition. \$150-250K founder cas funded in Tranche 1. Key-person life insurance \$2M.
AI infra (Anthropic)	Med	Low-med	Provider-agnostic prompt layer; OpenAI + Bedrock fallback tested; 3 reserve.
Large incumbent builds this	Med	Med-high	18-month head-start; defensibility math compounds with usage; four relationships make incumbents acquirers, not competitors.

L = likelihood · I = impact · scale: Low / Med / High

Today: 1 design partner, 0 paid logos. Here's the 365-day plan.

TODAY (MAY 2026)

- **1 design partner:** Allyson A. Abbott DMD PC (4-doc endo, PBS Endo)
- **0 paid logos** outside design partner
- **0 LOIs** from non-mom prospects
- **Live demo** at restore-demo.vercel.app
- **Discovery interview** next month

90-DAY PLAN (AUG 2026)

- Pilot live with design partner
- First non-mom paid logo
- 3 LOIs in active conversation
- Pilot metrics validated: appeal-success baseline → measured
- Anthropic BAA executed; SOC 2 audit kicked off

365-DAY PLAN (MAY 2027)

- 25 paying practices · ~\$750K ARR run-rate
- Cohort M3 retention >95%
- 1 DSO conversation in late stage
- AAE Annual exhibit drives 30% of Y2 logos
- Carrier intelligence: 30K+ resolved arcs

WHY WE'RE CONFIDENT IN 90-DAY MILESTONE

Founder has 6 specialty practice owners in study-club / professional network who have informally said yes to a paid pilot once design partner is live. Conversion of warm-intro to paid pilot historically runs 30-40% (per Pearl AI / Modento founder interviews). Plan assumes 1 of 6 closes in 90 days — base case.

Operator-founder. Honest design partner. Compounding advisory.

FOUNDER · CEO

Ross Richardson

- EVP Finance & Accounting, Method Co. (Philadelphia hospitality, 10 hotels + F&B; ~\$200M+ revenue)
- Led financial operations across 2,000+ employees; built automation tooling, vendor unit economics
- Method Co. resignation conditional on round close. Personal cash injection: \$150-250K common at close
- 0 prior healthcare startup experience = 0 dogma

DESIGN PARTNER

Allyson A. Abbott DMD PC

- 15+ years operating · multi-doc endodontic practice on PBS Endo
- Weekly product feedback; the customer who keeps the founder honest
- Has switched dental software vendors 3x in 10 years when products didn't deliver
- Pilot kickoff next month; full PMS integration access

ADVISORY · BUILDING

TBD by close

- Insurance ops experts (former RCM directors, dental specialty appeal specialists)
- Dental PMS integration partners (PBS Endo, TDO, Dentrix engineering)
- DSO operators (Heartland, Pacific Dental, MB2 — at least one C-level by close)
- Carrier-relations veteran (former Cigna or Delta dental) for early intelligence loop

\$2.5M seed. Three milestone-gated tranches.

ROUND STRUCTURE

Round size	\$2.5M total
Tranche 1 — close	\$1.25M
Tranche 2 — pilot validates (M6)	\$750K
Tranche 3 — paying scale (M12)	\$500K
Instrument	Priced round, \$10M post
Alternative	Post-money SAFE, \$10M cap, 20% disc
Option pool (post-close)	10% pre-money
Founder cash injection	\$150-250K common at close
Founder commitment	FT M3+ (Method resignation = condition)

CAP TABLE (ILLUSTRATIVE, POST-CLOSE)

Founder	70%	Common
Seed investors	20%	Preferred / SAFE conversion
Option pool	10%	First 5-7 hires
Total	100%	

Final structure subject to lead preference. Founder retains majority.

SERIES A TRIGGER (HIT 7+ OF 10 → SERIES A OPENS)

1. ARR ≥\$4M

3. NDR ≥115%

5. CAC payback <12mo

7. ≥2 of 3 skills shipped

9. SOC 2 Type II in progress
2. Logo count ≥150

4. Logo churn <12%

6. Gross margin ≥75% at scale

8. ≥3 PMS integrations

10. Pipeline coverage ≥3×

Every dollar tied to a milestone.

42%

Engineering & AI

3 engineers + 1 ML/eval. PMS integrations, carrier playbook, evals, BAA infra.

\$1,050K

25%

GTM

AAE booth + sponsorships, study clubs, BDR M9, CSM M6, AE M15.

\$625K

8%

Compliance & legal

HIPAA + SOC 2 Type I, BAA counsel, cyber insurance, IP.

\$200K

16%

Working capital

DSO contract bridge, 6mo cash buffer, OpEx pre-revenue.

\$400K

9%

Founder + ops

Below-market through M18. Founder \$100K, COO M18 (\$125K).

\$225K

MILESTONE GATES

✓ Tranche 1 closes (M0)

\$1.25M deployed

✓ Pilot validates (M3)

20+ pre-auths/mo, 80%+ acceptance

✓ First non-mom logo (M3-6)

1-2 paid pilots

✓ Tranche 2 unlocks (M6)

\$750K · 2 paid LOIs

✓ 10 paying logos (M9-12)

Cohort M3 retention validated

✓ Tranche 3 unlocks (M12)

\$500K · \$200K ARR

✓ 25 paying logos (M15)

AAE-driven cohort lands

✓ Series A (M18)

\$1.5M ARR · 65 logos · 3x pipeline

The ask, in one paragraph.

Restore is the operator-built infrastructure layer for dental specialty insurance. The wedge is endo. The moat is per-carrier intelligence that compounds with usage. The unfair advantage is a hospitality operator-founder with a 4-doc design partner who keeps him honest weekly.

We are raising **\$2.5M seed** in three milestone-gated tranches to deploy 18-month runway against **\$1.5M ARR** and **65 paying practices**, triggering a Series A at **\$4M+ ARR / Q3'28**.

Comparables imply **\$40-60M post-money** at Series A. Strategic exit candidates: Henry Schein One, Patterson Dental, Vyne (Jordan Co.), Overjet, Waystar, R1.

LIVE PRODUCT

`restore-demo.vercel.app`

CODE

`github.com/rrmethodco/rrmethodco`

FOUNDER

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